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TCGA-IM-A3U3-01A-PR

Redacted



and Clinics Laboratory

Surgical Pathology Report

Name:
DOB: (Age
Gender:
MRN:
Location:
Physician:

Case #:
Collected:
Received:
Reported:
Copy To:

Pathologic Interpretation:

A. RIGHT LOBE OF THYROID, 9.6 GRAMS:

PAPILLARY THYROID CARCINOMA, TALL CELL VARIANT, WITH SCLEROSIS, 1.3 cm in the mid pole.

The tumor approaches less than 1 cm from the inked margin.

Lymphovascular invasion is present.

PAPILLARY THYROID MICROCARCINOMA, FOLLICULAR VARIANT, WITH SCLEROSIS, 0.3 cm, inferior pole.

PAPILLARY THYROID MICROCARCINOMA, FOLLICULAR VARIANT, < 1 cm, inferior pole.

Multinodular hyperplasia.

Mild lymphocytic thyroiditis.

One lymph node, negative for metastatic carcinoma (0/1).

No parathyroid gland present.

B. LEFT LOBE OF THYROID, 16 GRAMS:

PAPILLARY THYROID MICROCARCINOMA, FOLLICULAR VARIANT, WITH SCLEROSIS, at least 5 foci, from 0.1 cm to 0.6 cm, throughout the left lobe.

Lymphovascular invasion is present.

Multinodular hyperplasia with focal adenomatous changes.

No parathyroid gland present.

No lymph nodes.

C. R/O PARATHYROID FS:

Minute fragment of lymph node.

No parathyroid gland present.

Negative for metastatic carcinoma.

ICD-O-3
carcinoma, papillary,
tall cell
8344/3
Site: thyroid, NOS
C73.9

4-10-12
20

D. RIGHT CENTRAL MIDLINE COMPARTMENT DISSECTION PERMANENT:

Eight lymph nodes, negative for metastatic carcinoma (0/8).

Thymus and fibroadipose tissue.

AJCC: pT1b, pN0, pMx (see Tumor Summary)

Surgical Pathology Cancer Case Summary

Procedure: Thyroid lobectomy (Right and Left)

Specimen Integrity: Intact

Specimen Size:

Right lobe: 5.5 x 2.3 x 1.0 cm

Left lobe: 5.0 x 3.0 x 2.0 cm.

Specimen Weight:

Right lobe: 9.6 grams

Left lobe: 16 grams

Tumor Focality:

Multifocal: Bilateral

Dominant Tumor:

Tumor Laterality: Right lobe

Tumor Size:

Greatest dimension: 1.3 cm

Histologic Type:

Papillary carcinoma

Variant, specify: Tall cell variant

Cytomorphology: Tall cell

Margins:

Margins uninvolved by carcinoma

Distance of invasive carcinoma to closest margin: < 1 mm

Tumor Capsule: None

Lymph-Vascular Invasion: Present: Extensive (4 or more vessels)

Perineural Invasion: Not identified

Extrathyroidal Extension: Not identified

Other Tumors:

Tumor Laterality (multifocal): Right and left lobes, multiple

Tumor Size:

Greatest dimension: from 0.1 cm to 0.6 cm

Histologic Type:

Papillary carcinoma

Variant, specify: Follicular variant with sclerosis

Architecture: Follicular

Cytomorphology: Classical

Margins: Margins uninvolved by carcinoma

Distance of invasive carcinoma to closest margin: 1 mm

Tumor Capsule: None

Lymph-Vascular Invasion:

Present: Extensive (4 or more vessels)

Perineural Invasion: Not identified

Extrathyroidal Extension: Not identified

Pathologic Staging (pTNM):

Primary Tumor (pT):

pT 1b: Tumor more than 1 cm but not more than 2 cm in greatest dimension, limited to the thyroid

Regional Lymph Nodes (pN):

pN0: No regional lymph node metastasis

Number examined: 8

Number involved: 0

Distant Metastasis (pM):

Not applicable

Additional Pathologic Findings:

Adenomatoid nodules or Nodular follicular disease (eg. nodular hyperplasia, goitrous thyroid)
Thyroiditis: Focal (nonspecific)

NOTE: Some immunohistochemical antibodies are analyte specific reagents (ASRs) validated by our laboratory (Her 2, Parvo, H. pylori, HBcore). These ASRs are clinically useful indicators that do not require FDA approval. These clones are used: ID5=ER, PgR 636=PR, A485=HER2, H-11=EGFR, CCH2/DDG9=CMV, F39.4.1=AR and HPV by ISH. All immunohistochemical stains are used with formalin or molecular fixed, paraffin embedded tissue. Detection is by IHC. The results are read by a pathologist as positive or negative.

Procedures/Addenda

Addendum

Date Ordered:

Status: Signed Out

Date Complete:

Date Reported:

Addendum Diagnosis

Immunohistochemistry performed at _____ reveal the following profile:

POSITIVE (A5-A13): IgG
IgG4 (20/hpf)
HBME1
Galectin 3

Ki67 proliferation index 5% (A5) and 3% (A13).

Immunohistochemistry performed at _____ reveal the following profile:

POSITIVE (B8,B11): IgG
IgG4 (3/HPF)
HBME1
Galectin 3

Ki67 proliferation index 2% (B8) and 10% (B11).

Intraoperative Consultation

CFS: Minute tissue fragment, possible parathyroid tissue. Dr. concurs. Communicated to surgeon.

Clinical History:

Patient is a _____ years old _____ with thyroid cancer

Pre Operative Diagnosis:

Not provided

Fee**Codes:**

A: Right lobe of thyroid (suture marks upper pole) (Fresh) H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1
B: Left lobe of thyroid -stich superior permanent H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1
C: R/O parathyroid fs Frozen section x 1, FS Perm x 1
D: Right central midline compartment dissection permanent H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1, H&E, Initial x 1

Gross Description:

A. Received in formalin labeled "Right lobe of thyroid suture mark upper pole" is a 9.6 grams of right lobe of thyroid measuring 5.5 x 2.3 x 1.0 cm. The external surface is inked in black. The parathyroid gland is not present grossly. Cut sections reveal grey-white well-defined tumor mass measuring 1.3 x 1.0 x 0.7 cm and less than 0.1 cm from the capsule, 2.0 cm from isthmus resection margin. This tumor is located in the mid pole. Toward inferior pole, there are multiple grey-tan cystic cavities measuring from 0.3 to 0.8 cm in greatest dimension. Specimen submitted in toto in 14 cassettes as follows:

Cassette #1: Resection margin

Cassettes #2-4: Superior pole

Cassettes #5-8: Mid pole, the lesion in toto

Cassettes #9-14: Inferior pole

B. Received in formalin labeled "Left lobe of thyroid stitch superior" is 16.0 grams of left lobe of thyroid measuring 5.0 x 3.0 x 2.0 cm. The external surface is inked in black. Parathyroid gland is not present. Cut sections reveal grey-tan well defined nodule (3.2 x 2.0 x 1.8 cm) with hemorrhagic and cysts, located at less than 0.1 cm from capsule and 0.5 cm from the isthmus resection margin. This nodule is extending from superior to inferior pole. Specimen submitted in toto in 12 cassettes as follows:

Cassettes #1-5: Superior pole

Cassettes #6-9: Mid pole

Cassettes #10-12: Inferior pole

C. Received fresh labeled "R/O parathyroid" is a grey-tan tiny fragment measuring 0.2 x 0.2 x 0.1 cm. Submitted in toto in 1 cassette for frozen section.

D. Received in formalin labeled "Right central midline compartment" is yellow fibroadipose tissue measuring 4.0 x 3.0 x 1.0 cm. There are 5 possible lymph nodes measuring up to 1.0 cm in greatest dimension. Specimen submitted in toto in 6 cassettes as follows:

Cassette #1: Largest bisected

Cassette #2,3: Two lymph nodes per cassette

Cassette #4-6: Adipose tissue

Criteria	Yes	No
Primary Discrepancy		///
Primary Tumor Site Discrepancy		///
Primary Site Discrepancy		///
Primary Management History		///
Pathologic/Immunohistochemistry/Genetic		///
Case is (circle) <u>QUALIFIED</u> <u>OR QUALIFIED</u>		
reviewer in this category		